



PROCEDURE

Discharge Planning for Adult Community Consumer

Scope (Staff):	All NMHS Mental Health (MH) Adult Community Staff
Scope (Area):	All NMHS MH Adult Community Areas

Contents

1. Aim	1
2. Background	1
3. Risk	1
4. Principles	2
5. Discharge Planning	2
6. Compliance, monitoring and evaluation	4
Appendix 1: NMHS MHPHDS Community Adult Discharge Checklist Sticker	8

1. Aim

All North Metropolitan Health Service Mental Health (NMHS MH) clinicians will actively plan the effective and timely discharge of consumers as per the [Department of Health \(DoH\) Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation](#), and legislative requirements of the [Mental Health Act](#) (MHA) 2014.

2. Background

This procedure has been developed to provide clinicians with clear and consistent direction as to the requirements regarding discharge of consumers. This gives consideration to the [DoH Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation](#) (SSCD), [National Safety and Quality Health Service Standards 2](#) (Partnering with Consumers), [Standard 6](#) (Communicating for Safety).

3. Risk

Failure of MH clinicians to actively plan the effective and timely discharge of their consumers can lead to future relapse and readmission of the consumer and lack of appropriate supports being put in place at the time of discharge.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐

Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐
Relapse and/or admission			

4. Principles

- All NMHS MH Community staff are to familiarise themselves with, and adhere to this procedure.
- Community Adult services are to use the available NMHS MH Community Adult Discharge Checklist Sticker (**Appendix 1**).
- Discharge planning, where possible and clinically indicated, is to occur in collaboration with the nominated person, carer, close family member (or other personal support person), GPs, other health services, and/or advocate/s to ensure effective transition between treatment settings (most commonly, community and community support services) to reduce the risk of relapse and ensure continuity of care. Written transfer of care is essential.
- Discharge planning is an integral part of the continuum of care that commences at the time of entry to community services and focuses on coordinating services effectively so that discharge occurs expediently and comprehensively such that all involved are appropriately engaged and informed.
- Consumers must be consulted and included in the decision as to the persons/agencies to whom information will be provided. Where a consumer has not given consent to information sharing the families can be given some general information to reassure them. If there are concerns about the release of information, this should be discussed with the consumer and the nominated person, primary carer/personal support person, and attempts made to resolve these concerns.

Note: If it is considered that there is significant risk to the consumer and/or others, then information may be provided to a nominated person, carer, close family member or other personal support person without explicit consumer consent where the clinician's duty of care takes priority over the patient's wishes

5. Discharge Planning

Discharge planning must be integral in the following stages of a consumer's transition through a community service:

- assessment
- treatment, care and support plan/management plan development
- treatment/intervention and outcome measures
- review and follow-up

Clear expectations of the potential length of treatment within community services must be provided on admission and during treatment. All clinicians are responsible for reviewing treatment needs and discharge planning.

The [Treatment, Support and Discharge Plan](#) (SMHMR907) / Management Plan is to start from assessment and is a continuum until discharge. It is developed with each consumer and the nominated personal support person, as per the [DoH Triage to Discharge Mental Health Framework for SSCD](#), and is utilised and updated throughout the consumer's admission to MH services. This document identifies discharge requirements from the point of admission and is utilised as the final document outlining discharge plans for the consumer prior to discharge from the service. The consumer and personal support person should be provided with the opportunity to sign this plan to indicate agreement (staff will document on the plan if either person refuses/is unable to sign and revisit this process at the earliest opportunity – maximum 3 monthly).

Requirements for ongoing care post discharge from the Community MH service are negotiated with the consumer and personal support person (unless declined) by the case manager / treating doctor. This may include linkage with appropriate agencies/services, which takes into account the cultural needs of the consumer (e.g. for Culturally and Linguistically Diverse, and Aboriginal and Torres Strait Islander consumers, culturally specific services should be considered and discussed with the consumer and personal support person). Where clinically indicated, the case manager/treating doctor should ensure that discussion has taken place with the GP prior to discharging all consumers.

Intent to discharge is presented and recorded at the multidisciplinary team (MDT) meetings with planning for ongoing care as appropriate. After the initial discussions further appointments should be available to the consumer to adjust to the pending discharge and to voice any concerns.

All discharges, including consumers who decline service, are discussed and reviewed by the Consultant Psychiatrist (Stokes 7.1). All clinicians should ensure they are familiar with, and follow the processes set out in the [NMHS MH Consumer Declining Assessment or Follow-up Policy](#), prior to discharging these consumers.

On discharge, the [Treatment, Support and Discharge Plan](#) (SMHMR907) / Management Plan is updated and discussed with the consumer and personal support person (Stokes 3.2 & 7.11.4). This should be signed by the consumer and personal support person to corroborate understanding of the plan (or date of discussion noted by clinician if refused). A copy of the plan is provided to the consumer, personal support person and GP, as appropriate (Stokes 2.2).

Ongoing care services (there may be multiple) are:

- Contacted to discuss the care transfer as agreed with the consumer.
- Provided with a care transfer summary or letter as detailed in the [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#).
- Provided with a copy of negotiated/signed [Treatment, Support and Discharge Plan](#) (SMHMR907) / Management Plan when appropriate, and as agreed with the consumer (Stokes 7.3).

Clinical records will be finalised on discharge by:

- Recording completion of all discharge requirements maintained in the medical records and for Community Adult utilising the NMHS MHPHDS Community Adult Discharge Checklist sticker (**Appendix 1**).

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- The Community Discharge Checklist should contain as a minimum:
 - National Outcome and Casemix Collection (NOCC) completion.
 - [Risk Assessment and Management Plan](#) (RAMP) (SMHMR 905) completion.
 - Identified that consumer and personal support person have been informed of discharge.
 - Provision of the [Treatment, Support and Discharge Plan](#)/Management Plan to the consumer and relevant others as agreed with the consumer.
 - A updated list of current medications including scripts provided.
 - Details of discharge address and phone number.
 - Details of communication with original referrer and/or GP.
 - Details of whom the discharge summary was provided to.
 - Filing a copy of the printed PSOLIS [Treatment, Support and Discharge Plan](#) (SMHMR907)/Management Plan
 - Filing a copy of the PSOLIS Discharge [RAMP](#)

The community service provides the consumer and personal support person/s with information on the process for facilitating re-entry for service and resources for crisis contact (*Stokes 7.11.4b*).

6. Compliance, monitoring and evaluation

The Medical Co-Director, MH Community is to demonstrate compliance with the established relevant clinical indicators relating to discharge.

Measures to be met are:

- 100% of active consumers have a completed [Treatment, Support and Discharge Plan](#) (SMHMR907) or Management Plan on PSOLIS and in the clinical file.
- 100% of discharge clinical records have completed discharge requirements and Community Adult Services have the *NMHS MHPHDS Community Adult Discharge Checklist sticker* in progress notes.
- There is a discharge letter or care transfer summary filed to GP or other service.
- 100% clinical records indicate involvement of and information provided to personal support person (or documentation of reasons why not informed).
- 100% of consumers have a discharge [RAMP](#) in file and on PSOLIS.
- 100% of active consumers have a discharge NOCC completed in PSOLIS and printed in file.

Related internal policies, procedures and guidelines

[DoH Clinical Handover Policy](#)

[DoH Safety Planning for Mental Health Consumers Policy](#)

[DoH Safety Planning for Mental Health Consumers Procedure](#)

[NMHS Clinical Documentation Policy](#)

[NMHS MH Case Management Procedure](#)

[NMHS MH Clinical Record Documentation Policy](#)

[NMHS MH Clinical Risk Assessment and Management \(CRAM\) Policy](#)

[NMHS MH Consumer Declining Assessment or Follow-up Policy](#)

[NMHS MH Did Not Attend \(DNA\) Community Appointment Procedure](#)

References

[Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#)

[Chief Psychiatrist's Standards for Clinical Care](#)

[DoH Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation](#)

[Mental Health Act 2014](#)

[National Safety and Quality Health Service Standards](#)

[Stokes B. Review of the admission or referral to and the discharge and transfer practices of public mental health facilities/services in Western Australia. Perth: Government of Western Australia. Mental Health Commission; 2012.](#)

[The Royal Australian & New Zealand College of Psychiatrists](#)

Useful resources (including related forms)

[Risk Assessment and Management Plan \(RAMP\) \(SMHMR905\)](#)

[Treatment, Support and Discharge Plan \(SMHMR907\)](#)

MHPHDS Community Adult Discharge Checklist Sticker

Policy Sponsor	Co-Director, Community Adult MH				
Policy Contact	Co-Director, Community Adult MH				
Date First Issued:	08/07/2016	Last Reviewed:	11/08/2023	Review Date:	11/08/2026
Approved by:	Co-Director, Community Adult MH			Date:	09/08/2023
Endorsed by:	Executive Director MHPHDS			Date:	11/08/2023
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☒ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
11/07/2016	1.0	A/Executive Director	Initial publication
09/09/2022	2.0	A/Co Director, Community Adult MH	Scheduled review, review and update of all MH Adult Community discharge processes, addition of the MHPHDS Community Adult Discharge Checklist Sticker, template and hyperlinks updated.
09/08/2023	2.1	Co-Director, Community Adult MH	Scheduled review, minor amendments and hyperlinks updated

15/01/2024	2.2	Policy Officer	Hyperlinks updated
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The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD 82).

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Appendix 1: NMHS MHPHDS Community Adult Discharge Checklist Sticker

MHPHDS Community Adult Discharge Checklist	
SERVICE REQUIREMENTS	
Notify	☐ Consumer & Support Person/Carer
	☐ Administration as per service requirements
Update	☐ CAPlan
	☐ RAMP
	☐ Care Plan
Complete	☐ Discharge NOCC (HoNOS, LSP-16, K10+)
	☐ Discharge entry in medical record
	☐ PSOLIS Admission, Discharge, Transfer & Referral Form
	☐ Care Transfer Summary (PMHMR916)
CONSUMER & SUPPORT PERSON/CARER REQUIREMENTS	
Provide	☐ Information regarding re-entering the service
	☐ Information regarding emergency services
	☐ Copy of CAPlan
	☐ Copy of Care Plan
	☐ Copy of Coping and Awareness Tool (optional)
	☐ Copy of Care Transfer Summary (PMHMR916) if appropriate
	☐ Medication information as required
	☐ Follow up arrangements as required
Notify original referrer and/or GP	
Or	☐ Letter / Phone Call
	☐ Copy of Care Transfer Summary (PMHMR916) if appropriate
Discharge address & phone number	
Completed by	
Signed	
Dated	